

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Prospective Registry Study to Assess Real-world Patient Characteristics, Treatment Patterns, and Longitudinal Outcomes in Patients Receiving Mavacamten and Other Treatments for Symptomatic Obstructive Hypertrophic Cardiomyopathy (Obstructive-HCM)

Official Title: Deliver Insights in Hypertrophic Cardiomyopathy and Observational Outcomes in Real World: United States and European Prospective Registry Study

Short Title/Acronym: DISCOVER-HCM

Protocol Number: CV027-012

NCT Number: NCT05489705

Posting ID: 400088017116238223

Trial Status: RECRUITING

Sponsor/Company Name: Bristol Myers Squibb

Investigational Product: Mavacamten and other non-mavacamten symptomatic oHCM therapies

Phase of Development: Not Applicable

Medical Monitor: Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate patient characteristics, real-world treatment patterns, and short- and long-term outcomes in a population of patients in the United States and Europe with symptomatic obstructive hypertrophic cardiomyopathy (HCM) who are receiving mavacamten, receiving other treatment for obstructive HCM, or not receiving treatment due to intolerance or failure of prior treatment. **Secondary Objectives:** * United States Sub-Study: To evaluate the safety of mavacamten in patients with symptomatic obstructive HCM in the real-world setting. * Europe Sub-Study: To evaluate the effectiveness and safety of mavacamten in patients with symptomatic obstructive HCM in the real-world setting.

2.2 Background & Rationale To address the gap in real-world evidence regarding the long-term effectiveness, safety, and treatment patterns of mavacamten in routine clinical practice. While conventional treatments like beta-blockers and calcium channel blockers focus on symptomatic control, mavacamten is a novel cardiac myosin inhibitor targeting the underlying hypercontractility at the sarcomere level. This registry gathers real-world longitudinal data to monitor both symptom improvement and potential adverse effects such as ejection fraction decline outside of strict clinical trial environments.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational / Patient Registry
Control Type: N/A (Registry evaluating various real-world treatment arms) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target **N**: 1,600 participants
Number of Sites: Multiple sites across the United States and Europe
Estimated Study Start: August 2022 **Estimated Primary Completion:** August 2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Diagnosis of obstructive HCM consistent with 2020 AHA/ACC guidelines, defined by increased LV wall thickness ≥ 15 mm (or ≥ 13 mm with positive family history of HCM) in a nondilated ventricular chamber not explained by abnormal loading conditions, and peak LVOT gradient of ≥ 30 mmHg at rest or with provocation. 3. Documented LVEF of $\geq 55\%$ recorded by echocardiography within the last 6 months. 4. Symptoms consistent with NYHA functional class II-IV. 5. Receiving beta blockers (BBs), non-dihydropyridine calcium channel blockers (non-DHP CCBs), disopyramide, and/or mavacamten as part of routine clinical care; or currently receiving no treatment due to intolerance or failure of prior treatment.

4.2 Exclusion Criteria 1. Known phenocopy disease (e.g., Fabry disease, amyloidosis) or LV hypertrophy associated with hypertension. 2. Documentation of any fixed obstruction of the outflow tract such as aortic valve stenosis or replacement. 3. Prior treatment of obstructive HCM with invasive septal reduction (surgical myectomy or percutaneous alcohol septal ablation) within 6 months prior to enrollment. 4. Naïve to treatment for obstructive HCM (i.e., never treated with BBs, nonDHP CCBs, or disopyramide).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Drug Name: Mavacamten **Trade Name:** Camzyos **ATC Code:** C01EB24 **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/camzyos-epar-product-information_en.pdf **Dosage/Frequency:** Mavacamten or other standard-of-care therapies (BBs, CCBs) administered per routine clinical care guidelines and physician prescription. **Route of Administration:** Oral **Duration of Treatment:** Long-term routine clinical care monitoring through the registry's completion (up to August 2029).

5.2 Concomitant & Prohibited Therapy Permitted: Beta-blockers, non-dihydropyridine calcium channel blockers, and disopyramide.

Prohibited: Receiving an investigational therapeutic agent for obstructive HCM in an interventional clinical trial at enrollment; previously or currently enrolled in specific long-term safety extension studies of mavacamten (e.g., EXPLORER-HCM, MAVA-LTE, VALOR-HCM).

6. Study Timeline & Visit Schedule

(Note: As an observational registry, visits align with routine clinical care) | Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -X to 0 | Informed Consent, Medical History, Echocardiogram (LVEF $\geq 55\%$) | | **Baseline** | Day 0 | Registry Enrollment, Document Baseline Therapy, NYHA Class | | **Interim** | Routine Care | Regular echocardiograms (e.g., 21-28 days, 4 weeks, and maintenance for mavacamten REMS compliance), Safety & Effectiveness Tracking | | **End of Study** | Aug 2029 | Final Registry Data Collection, Longitudinal Outcomes Assessment |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Assessment of real-world patient characteristics, treatment patterns, and longitudinal short- and long-term clinical outcomes in patients with symptomatic oHCM.
Measurement Method: Registry data extraction from routine clinical care records and echocardiographic assessments.

7.2 Secondary & Exploratory Endpoints Real-world safety of mavacamten (United States Sub-Study). Real-world effectiveness and safety of mavacamten (Europe Sub-Study). Monitoring of left ventricular ejection fraction (LVEF) maintenance and echocardiographic structural changes.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard observational monitoring; for mavacamten, strict echocardiographic monitoring is mandated (e.g., via the REMS program) to monitor for potential detrimental inotropic impacts, particularly drops in ejection fraction $< 50\%$. **Serious Adverse Events (SAEs):** Routine pharmacovigilance and registry SAE reporting mechanisms for heart failure symptoms, atrial fibrillation, and stroke. **Data Monitoring Committee (DMC):** Overseen by the Sponsor and steering committee investigators.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Real-World Data (RWD) Registry cohort.
Sample Size Justification: Targeted enrollment of $N=1,600$ to

adequately capture diverse and representative real-world outcomes and track low-frequency adverse events across the US and Europe. **Handling of Missing Data:** Standard epidemiological multiple imputation and observational analysis methods suitable for RWD registries.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This registry will be conducted in accordance with Good Clinical Practice (GCP), real-world data guidelines, and the Declaration of Helsinki. **Monitoring Plan:** Periodic remote and onsite auditing of clinical data extraction from participating sites. **Audit Trail:** Electronic Data Capture (EDC) systems utilized for secure patient registry logging and eCRF completion.

11. Study Identifiers & Governance

Primary ID: {{CV027-012}} **Registry Number:** {{NCT05489705}}
Posting ID: {{400088017116238223}} **Sponsor/Lead Organization:** {{Bristol Myers Squibb}} **Study Title:** {{DISCOVER-HCM}} **Official Regulatory Title:** {{Deliver Insights in Hypertrophic Cardiomyopathy and Observational Outcomes in Real World: United States and European Prospective Registry Study}}

12. Clinical Framework (oHCM Specific)

Disease Area: {{Obstructive Hypertrophic Cardiomyopathy}}
Preferred UMLS Name: {{Hypertrophic Obstructive Cardiomyopathy}}
Primary Condition: {{Symptomatic Obstructive Hypertrophic Cardiomyopathy (oHCM)}} **Diagnosis Threshold:** {{LV wall thickness ≥ 15 mm, LVOT gradient ≥ 30 mmHg}} **Medical Methodology:** {{Guideline-directed routine clinical care & Cardiac Myosin Inhibition}} **Optimization Target:** {{Symptom resolution and LVEF preservation $\geq 50\%$ }}

13. Study Procedures & Methodology

Management Pathway: {{Real-world routine clinical care pathway for oHCM}} **Education Protocol (HCP):** {{REMS program compliance and safety monitoring for mavacamten}} **Education Protocol (Patient):** {{Disease education and mandatory echocardiogram scheduling}} **Quality Audit Mechanism:** {{Longitudinal registry data extraction and real-world compliance tracking}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 7 years (Aug 2022 to Aug 2029)}} **Onsite Visit Cadence:** {{Determined by routine clinical care and Mavacamten REMS requirements}} **Remote Monitoring Frequency:** {{As per standard registry data collection protocol}} **Checklist Review Interval:** {{Periodic registry abstraction reviews}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					